Thyrocare
Tests you can trust

Name : Reniguntla Venkata Subbalakshmi(72Y/F)

Date : 4 May, 2026

Test Asked : Complete Health Check For Couple With Vitamins, Fbs + 2 Others

Report Status : Complete Report

Processed At :
H. NO. 1-9-645, Vidyanagar,
Adikmet Road, Near SBH,
Hyderabad-500044

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Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703. 9870666333 wellness@thyrocare.com

First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation#

Patient Name	: **RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)**	Tests Done	: **COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS,FBS,PPBS,TROPONIN I SE**
Referred By	: **SELF**		
Address	: **G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090**		

Report Availability Summary

Note: Please refer to the table below for status of your tests.

19 Ready   **0** Ready with Cancellation   **0** Processing   **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
POSTPRANDIAL BLOOD SUGAR(GLUCOSE)	Ready
FASTING BLOOD SUGAR(GLUCOSE)	Ready
TROPONIN I SE	Ready
COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS	Ready
CARCINO EMBRYONIC ANTIGEN (CEA)	Ready
CHLORIDE	Ready
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready
Lipoprotein (a) [Lp(a)]	Ready
SODIUM	Ready
COMPLETE URINE ANALYSIS	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
LIVER FUNCTION TESTS	Ready
ELEMENTS 22 (TOXIC AND NUTRIENTS)	Ready
IRON DEFICIENCY PROFILE	Ready
KIDPRO	Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready
VITAMIN D TOTAL AND B12 COMBO	Ready
APOLIPROTEIN RATIO	Ready

Processed At :
D-37/1,TTC MIDC,Turbhe, Navi
Mumbai - 400703

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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation#

Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)		Tests Done	: COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS,FBS,PPBS,TROPONIN I SE
Referred By	: SELF			
Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090			

Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
HEMOGLOBIN	**11.7**	g/dL	12.0-15.0
MEAN CORP.HEMO.CONC(MCHC)	**29.5**	g/dL	31.5-34.5
MONOCYTES - ABSOLUTE COUNT	**0.13**	X 10^3 / µL	0.2 - 1.0
RED CELL DISTRIBUTION WIDTH (RDW-CV)	**14.6**	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	**53.3**	fL	39.0-46.0
COMPLETE URINE ANALYSIS			
SPECIFIC GRAVITY	**< 1.003**	-	1.003-1.030
ELECTROLYTES			
CHLORIDE	**107.34**	mmol/L	98 - 107
LIVER			
SGOT / SGPT RATIO	**2.15**	Ratio	< 2
RENAL			
EST. GLOMERULAR FILTRATION RATE (eGFR)	**81**	mL/min/1.73 m2	>= 90
VITAMINS			
25-OH VITAMIN D (TOTAL)	**18.1**	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

Processed At :
H. NO. 1-9-645, Vidyanagar,
Adikmet Road, Near SBH,
Hyderabad-500044

MC-5241

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Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	: 04 May 2026 07:19
Referred By	: SELF	Sample Received on (SRT)	: 04 May 2026 13:26
Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090	Report Released on (RRT)	: 4 May 2026 15:52
		Sample Type | Barcode	: FLUORIDE PLASMA | EZ982356

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	84.06	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.
Method:- GOD-POD METHOD

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

Dr Amulya MD (Path)

Dr Abdur R MD (Biochem)

Scan QR to verify(valid for 30 days from release time)

Processed At :
H. NO. 1-9-645, Vidyanagar,
Adikmet Road, Near SBH,
Hyderabad-500044

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Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	: 04 May 2026 07:19
Referred By	: SELF	Sample Received on (SRT)	: 04 May 2026 15:13
Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090	Report Released on (RRT)	: 4 May 2026 17:20
		Sample Type | Barcode	: URINE | FL605014

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
Physical Examination				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	**pKa change**	**< 1.003**	**-**	**1.003-1.030**
PH	pH indicator	7.5	-	5-8
Chemical Examination				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
Microscopic Examination				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-2
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	1	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

Tests Done : **BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS**

Amulya

Dr Amulya MD (Path)

Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	: 04 May 2026 10:57
Referred By	: SELF	Sample Received on (SRT)	: 04 May 2026 18:01
Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090	Report Released on (RRT)	: 4 May 2026 18:58
		Sample Type | Barcode	: FLUORIDE PLASMA | FM192631

TEST NAME	TECHNOLOGY	VALUE	UNITS
POSTPRANDIAL BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	84.37	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Random/Post-Prandial Plasma Glucose (RPG/PPPG)	
Normal	70 to 140 mg/dl
Impaired Glucose Tolerance	140 - 199 mg/dl
Diabetes	Greater than or Equal to 200 mg/dl

Please correlate with clinical conditions.
Method:- GOD-POD METHOD

Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	:	04 May 2026 07:19
Referred By	: SELF	Sample Received on (SRT)	:	05 May 2026 05:42
Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090	Report Released on (RRT)	:	5 May 2026 10:56
		Sample Type | Barcode	:	SERUM | FA432062

TEST NAME	TECHNOLOGY	VALUE	UNITS
CARCINO EMBRYONIC ANTIGEN (CEA)	E.C.L.I.A	5.68	ng/mL

Bio. Ref. Interval. :-

Non Smokers (Past / Never Smoked) - <5
Smokers (current) - <6.5

Clinical Significance:

1. CEA is often used to monitor patients with cancers of the gastrointestinal tract (GI). Increased CEA levels can indicate some Non-Cancer related conditions, Such as some forms of inflammation, Cirrhosis, and Peptic Ulcer. Also, Smokers tend to have Higher CEA levels than Non-Smokers.
When cancer spreads to other organs, CEA levels rise and may be present in other types of bodily fluids besides blood.
2. Samples should not be taken from patients receiving therapy with high biotin doses (i.e >5 mg/day) until atleast 8 hrs following the last biotin administration, as this may interfere with the result.
3. In few cases, interference due to extremely high titres of antibodies to analyte - specific antibodies, streptavidin or ruthenium can occur.
4. For Diagnostic Purpose, Results should always be assessed in Conjunction with the patients medical history, clinical examination and other findings.

References
- Thompson J, Zimmermann W. The carcinoembryonic antigen gene family: strcuture, expression and evolution. Tumour Biol 1988; 9(2-3):63-83
- Kit insert

Please correlate with clinical conditions.
Method:- Fully Automated Electrochemiluminescence Sandwich Immunoassay

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

Dr Arshiya MD(Path)

Dr Ranjana Roy MD(Path)

Processed At :
D-37/1,TTC MIDC,Turbhe, Navi
Mumbai - 400703

MC-2407

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Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090	Report Released on (RRT)	: 5 May 2026 10:56
		Sample Type | Barcode	: SERUM | FA432062

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**E.C.L.I.A**	**18.1**	**ng/mL**

Bio. Ref. Interval. :
Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

Method : Fully Automated Electrochemiluminescence Compititive Immunoassay

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	E.C.L.I.A	403	pg/mL

Bio. Ref. Interval. :
Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Method : Fully Automated Electrochemiluminescence Compititive Immunoassay

Please correlate with clinical conditions.

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

Dr Arshiya MD(Path)

Dr Ranjana Roy MD(Path)

Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	: 04 May 2026 07:19
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		Sample Type | Barcode	: SERUM | FA432062

TEST NAME	TECHNOLOGY	VALUE	UNITS
TROPONIN I HEART ATTACK RISK	C.M.I.A	0.25	pg/mL

Bio. Ref. Interval. :-

Cutoff values (Precision at 99th percentile) : Male : <= 26.2 || Female : <= 15.6
Clinical Significance:
The Cardiac Troponin I is cardiac specific and highly sensitive marker for myocardial damage. In acute myocardial damage the Troponin I values are raised 4-6 hrs after cardiac symptom onset and may remain elevated for a period of 7-12 days of cardiac injury. It is an independent risk marker that can predict near, mid and long term outcome in patients with Acute coronary syndrome. A single Troponin I result may not be sufficient to evaluate MI. Serial Blood draws are recommended to evaluate Acute coronary syndrome (ACS) patients.Increased levels of Cardiac troponin I can be seen in Myocarditis,heart contusion, cardiomyopathy,congestive heart failure,interventional therapy like cardiac surgery and drug induced cardiomyopathy. Any condition resulting in myocardial injury can potentially increase Troponin I levels.
Hence The results should always be used in conjunction with ECG changes, Clinical symptoms, other findings etc. to Diagnose MI. The various interfering factors / limitations of the Troponin I assay are - presence of heterophile antibodies, Patients on Human anti mouse monoclonal antibody (HAMA) therapy, Rheumatoid factor (RF), high total protein levels etc. The Coronary vascular disease risk stratification in Asymptomatic patients is as given in the below table, which can be suggested for preventive clinical management in conjunction with other clinical findings, investigations and clinical symptoms.

The following cut-off points may be used to aid in stratifying the risk of cardiovascular disease in asymptomatic individuals.

HIGH SENSITIVE TROPONIN-I LEVEL		
MALE	**FEMALE**	**INTERPRETATION**
<6	<4	Low risk of future heart attack
≥6 to ≤12	≥4 to ≤10	Moderate risk of future heart attack
>12	>10	Elevated risk of future heart attack

Specifications: Precision at the 99th Percentiles
Females = 15.6 pg/mL -5.3% CV || Males = 34.2pg/mL - 3.5% CV

Kit validation reference :
1. Risk-Stratification of the Apparently Healthy Population for Future Cardiac Events With ARCHITECT High Sensitive Troponin-I(https://dam.abbott.com/de-de/documents/pdf/ADD- 00064393-hsTnI-Risk-Stratification-Sell-Sheet.pdf)
2. ARCHITECT STAT High Sensitive Troponin-I [package insert]. Lake Bluff, IL: Abbott Laboratories; 2018. G97079R01.

Please correlate with clinical conditions.
Method:- FULLY AUTOMATED CHEMI LUMINESCENT MICROPARTICLE IMMUNOASSAY

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

Dr Arshiya MD(Path)

Dr Ranjana Roy MD(Path)

Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	: 04 May 2026 07:19
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		Sample Type | Barcode	: SERUM | FA432062

TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1)	IMMUNOTURBIDIMETRY	123	mg/dL

Bio. Ref. Interval. :
120 - 190

Method : Immunoturbidometric test

TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - B (APO-B)	IMMUNOTURBIDIMETRY	70	mg/dL

Bio. Ref. Interval. :
Male : 56 - 145
Female : 53 - 138

Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

TEST NAME	TECHNOLOGY	VALUE	UNITS
APO B / APO A1 RATIO (APO B/A1)	CALCULATED	0.6	Ratio

Bio. Ref. Interval. :

Male : 0.40 - 1.26
Female : 0.38 - 1.14

Clinical Significance :

- Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol.
- Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD.
- The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc.
- An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk.
- All results have to be interpreted in Conjunction with clinical history and other findings.

Method : Derived from serum Apo A1 and Apo B values

Please correlate with clinical conditions.

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

Dr Arshiya MD(Path)

Dr Ranjana Roy MD(Path)

Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	**Sample Collected on (SCT)**	: 04 May 2026 07:19
Referred By	: SELF	**Sample Received on (SRT)**	: 05 May 2026 05:42
Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090	**Report Released on (RRT)**	: 5 May 2026 10:56
		Sample Type | Barcode	: SERUM | FA432062

TEST NAME	TECHNOLOGY	VALUE	UNITS
LIPOPROTEIN (A) [LP(A)]	IMMUNOTURBIDIMETRY	3.6	mg/dL

Bio. Ref. Interval. :-

Adults : < 30.0 mg/dl

Clinical Significance:
Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:
Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:
Tietz NW,Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.
Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

Dr Arshiya MD(Path)

Dr Ranjana Roy MD(Path)

Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	:	04 May 2026 07:19
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Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090	Report Released on (RRT)	:	5 May 2026 10:56
		Sample Type | Barcode	:	SERUM | FA432062

TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	IMMUNOTURBIDIMETRY	1.46	mg/L

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

Clinical significance:
High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:
1.Clinical management of laboratory date in medical practice 2003-3004, 207(2003).
2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.
Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	: 04 May 2026 07:19
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		Sample Type | Barcode	: SERUM | FA432062

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON **Bio. Ref. Interval. :** 37 - 165 **Method :** Photometric test using Ferene	PHOTOMETRY	67.2	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) **Bio. Ref. Interval. :** Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl **Method :** Spectrophotometric Assay	PHOTOMETRY	377.4	µg/dL
% TRANSFERRIN SATURATION **Bio. Ref. Interval. :** 13 - 45 **Method :** Derived from IRON and TIBC values	CALCULATED	17.81	%
UNSAT.IRON-BINDING CAPACITY(UIBC) **Bio. Ref. Interval. :** 162 - 368 **Method :** SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	310.2	µg/dL

Please correlate with clinical conditions.

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

Dr Arshiya MD(Path)

Dr Ranjana Roy MD(Path)

Processed At :
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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	160	mg/dL	<200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	51	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	81	mg/dL	<100
TRIGLYCERIDES	PHOTOMETRY	118	mg/dL	<150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	3.2	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	2.34	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	1.6	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.62	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	109	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	23.66	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - "CHOD-PAP" enzymatic photometric test
HCHO - Direct HDL measurement by Homogenous Method
LDL - Direct LDL measurement by Homogenous Method
TRIG - Colorimetric GPO
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

Dr Arshiya MD(Path)

Dr Ranjana Roy MD(Path)

Processed At :
D-37/1,TTC MIDC,Turbhe, Navi
Mumbai - 400703

MC-2407

Thyrocare
Tests you can trust

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Patient Name	: RENIGUNTLA VENKATA SUBBALAKSHMI(72Y/F)	Sample Collected on (SCT)	:	04 May 2026 07:19
Referred By	: SELF	Sample Received on (SRT)	:	05 May 2026 05:42
Address	: G3 KSR SAI DINESH ARCADE ROAD NO:10C BHANDARI LAYOUT NIZAMPET HYDERABAD PIN CODE : 500090	Report Released on (RRT)	:	5 May 2026 10:56
		Sample Type | Barcode	:	SERUM | FA432062

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	54.32	U/L	33-98
BILIRUBIN - TOTAL	PHOTOMETRY	0.45	mg/dL	0.10 - 1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.19	mg/dL	< 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.26	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	11.4	U/L	<40
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	19.8	U/L	< 31
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	9.2	U/L	<34
SGOT / SGPT RATIO	**CALCULATED**	**2.15**	**Ratio**	**< 2**
PROTEIN - TOTAL	PHOTOMETRY	6.9	gm/dL	6.6 to 8.3
ALBUMIN - SERUM	PHOTOMETRY	4.1	gm/dL	3.5 - 5.2
SERUM GLOBULIN	CALCULATED	2.8	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.46	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Kinetic Photometric test as per Modified IFCC
BILT - Photometric test using DCA (2,4 Dichloroaniline)
BILD - Photometric test using DCA (2,4 Dichloroaniline)
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Kinetic Photometric test Szasz/Persijn standardized as per IFCC
SGOT - Optimized UV Test IFCC modified
SGPT - Optimized UV test as per IFCC Modified
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Photometric test using Bromocresol Green
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS
SODIUM	I.S.E - INDIRECT	142.89	mmol/L

Bio. Ref. Interval. :
ADULTS: 136-145 MMOL/L

Method : ION SELECTIVE ELECTRODE - INDIRECT

CHLORIDE	**I.S.E - INDIRECT**	**107.34**	**mmol/L**

Bio. Ref. Interval. :
ADULTS: 98-107 MMOL/L

Clinical Significance :
An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).

Method : ION SELECTIVE ELECTRODE - INDIRECT

Please correlate with clinical conditions.

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	13.2	mg/dL	7.94 - 20.1
CREATININE - SERUM	PHOTOMETRY	0.78	mg/dL	0.51 - 0.95
BUN / SR.CREATININE RATIO	CALCULATED	16.92	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	28.25	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	36.22	Ratio	< 52
URIC ACID	PHOTOMETRY	4.43	mg/dL	2.3-6.1
CALCIUM	PHOTOMETRY	9.04	mg/dL	8.6 - 10.3

Please correlate with clinical conditions.

Method :

BUN - "Urease-GLDH": Enzymatic UV test
SCRE - Enzymatic colorimetric test
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
URIC - Enzymatic Photometric test using TOOS
CALC - Photometric test using Arezano III

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	85	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	6.55	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	4.58	µIU/mL	0.54-5.30

Comments : SUGGESTING THYRONORMALCY

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	**CALCULATED**	**81**	**mL/min/1.73 m2**

Bio. Ref. Interval. :-

\> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.
Method:- 2021 CKD EPI Creatinine Equation

Tests Done : BLOOD SUGAR (F),BLOOD SUGAR (PP),TROPONIN I SE,COMPLETE HEALTH CHECK FOR COUPLE WITH VITAMINS

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			Sample Type | Barcode	: EDTA Whole Blood | FA286448

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ARSENIC	ICP-MS	0.49	µg/L	< 5
CADMIUM	ICP-MS	0.73	µg/L	< 1.5
MERCURY	ICP-MS	0.27	µg/L	< 5
LEAD	ICP-MS	42.83	µg/L	< 150
CHROMIUM	ICP-MS	0.85	µg/L	< 30
BARIUM	ICP-MS	2.48	µg/L	< 30
COBALT	ICP-MS	0.57	µg/L	0.10 - 1.50
CAESIUM	ICP-MS	2.05	µg/L	< 5
THALLIUM	ICP-MS	0.05	µg/L	< 1
URANIUM	ICP-MS	0.04	µg/L	< 1
STRONTIUM	ICP-MS	28.21	µg/L	8 - 38
ANTIMONY	ICP-MS	13.13	µg/L	0.10 - 18
TIN	ICP-MS	0.35	µg/L	< 2
MOLYBDENUM	ICP-MS	2.26	µg/L	0.70 - 4.0
SILVER	ICP-MS	1.43	µg/L	< 4
VANADIUM	ICP-MS	0.32	µg/L	< 0.8
BERYLLIUM	ICP-MS	0.06	µg/L	<0.10
BISMUTH	ICP-MS	0.36	µg/L	0.10 - 0.80
SELENIUM	ICP-MS	102.96	µg/L	60 - 340
ALUMINIUM	ICP-MS	18.88	µg/L	< 30
NICKEL	ICP-MS	1.33	µg/L	< 15
MANGANESE	ICP-MS	19.24	µg/L	7.10 - 20

Please correlate with clinical conditions.

Method :

ICP - MASS SPECTROMETRY
Note:Reference range has been obtained after considering 95% population as cutoff.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5.6	%

Bio. Ref. Interval. :

As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

TEST NAME	TECHNOLOGY	VALUE	UNITS
AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	114	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	**SLS-Hemoglobin Method**	**11.7**	**g/dL**	**12.0-15.0**
Hematocrit (PCV)	CPH Detection	39.7	%	36.0-46.0
Total RBC	HF & EI	4.08	X 10^6/µL	3.8-4.8
Mean Corpuscular Volume (MCV)	Calculated	97.3	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	28.7	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	**Calculated**	**29.5**	**g/dL**	**31.5-34.5**
Red Cell Distribution Width - SD (RDW-SD)	**Calculated**	**53.3**	**fL**	**39.0-46.0**
Red Cell Distribution Width (RDW - CV)	**Calculated**	**14.6**	**%**	**11.6-14.0**
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	348.2	-	*Refer Note below
MENTZER INDEX	Calculated	23.8	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	4.5	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	54.4	%	40-80
Lymphocytes Percentage	Flow Cytometry	37.1	%	20-40
Monocytes Percentage	Flow Cytometry	2.9	%	2-10
Eosinophils Percentage	Flow Cytometry	4.7	%	1-6
Basophils Percentage	Flow Cytometry	0.7	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0.0-0.4
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	2.45	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	1.67	X 10^3 / µL	1.0-3.0
Monocytes - Absolute Count	**Calculated**	**0.13**	**X 10^3 / µL**	**0.2 - 1.0**
Basophils - Absolute Count	Calculated	0.03	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.21	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.01	X 10^3 / µL	0.0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	205	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	11	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	12.5	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	31.2	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.22	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

~~ End of report ~~

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Dr Arshiya MD(Path)

Dr Ranjana Roy MD(Path)

Scan QR to verify(valid for 30 days from release time)

CONDITIONS OF REPORTING

- v The reported results are for information and interpretation of the referring doctor only.
- v It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- v Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- v Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- v Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- v This report is not valid for medico-legal purpose.
- v Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- v Thyrocare Discovery video link :- https://youtu.be/nbdYeRgYyQc

EXPLANATIONS

- v Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- v **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- v **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- v **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- v **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- v **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- v **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- v **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- v **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- v **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- v Values out of reference range requires reconfirmation before starting any medical treatment.
- v Retesting is needed if you suspect any quality shortcomings.
- v Testing or retesting should be done in accredited laboratories.
- v For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**

+ *T&C Apply, #As on 5th December 2024 (Applicable for all company owned labs except Bhagalpur & Vijayawada),*

* *As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited*